BASELINE → TREATMENT → FOLLOW-UP

# When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

| Check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
|---|---|---|---|---|---|
| **Blood count, kidney and liver** | Baseline blood count, kidney and liver profile | Review results and symptoms before treatment | Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts | Repeat sooner if unwell or results need follow-up | Follow unresolved abnormalities after treatment |
| **Magnesium, calcium and potassium** | Check mineral levels before starting | Repeat periodically during treatment | Keep blood-test and replacement appointments | More checks may be needed if levels fall or diarrhoea develops | Monthly checks for two months after the last antibody dose |
| **Skin, nails, eyes and breathing** | Discuss existing skin, eye or lung problems | Start the prescribed skin-care plan; report reactions early | Review skin, nails, diarrhoea and new symptoms | Eye pain, worsening breathing or severe rash needs prompt assessment | Continue care until symptoms and mineral levels settle |
| **Bowel symptoms and hydration** | Have a written diarrhoea plan and emergency contact | Tell the nurse about cramps, sweating or diarrhoea during infusion | Delayed diarrhoea can start after the first day; act using your written plan | Fever, reduced urine, severe pain or inability to drink needs urgent advice | Restart decisions require recovery and team review |
| **Fluorouracil safety and line** | Confirm the DPD test and safe line/pump arrangements | Check mouth, bowel and pump symptoms early | Review line, pump and previous-cycle toxicity | Report chest pain or unexpectedly severe early side effects immediately | Confirm disconnection and ongoing line care |
| **Cancer response and treatment choices** | Record baseline disease and agree review plan | Bring forward review if symptoms worsen | Scans and other tests follow your consultant's treatment plan | Discuss benefit, side effects and whether the goal has changed | Agree continuation, a break, another treatment or supportive care |
| **Conditional heart assessment** | ECG if cardiac function is compromised | Report new chest pain or breathing change | Emergency assessment for chest pain, collapse or severe breathlessness | Additional cardiac tests when clinically indicated | Follow any identified problem; no routine fixed echo series |
| **INR if taking warfarin** | Tell the team about warfarin | Check INR as clinically indicated | Report bleeding and major changes in food intake | Warfarin doses are adjusted by the prescribing team | Continue the individually agreed anticoagulation plan |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00585 v5b · current official regimen checked 5 November 2026 · EMA: Erbitux product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

# Fortnightly cetuximab + FOLFIRI: treatment and checks

### Advanced / metastatic colorectal cancer

Cetuximab and FOLFIRI start on Day 1, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat every 14 days while beneficial and tolerable. No weekly Day 8 cetuximab is scheduled.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
|---|---|---|---|---|---|
| **Blood count, kidney and liver** NCCP 00585 | Baseline blood count, kidney and liver profile | Review results and symptoms before treatment | Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts | Repeat sooner if unwell or results need follow-up | Follow unresolved abnormalities after treatment |
| **Magnesium, calcium and potassium** NCCP 00585 | Check mineral levels before starting | Repeat periodically during treatment | Keep blood-test and replacement appointments | More checks may be needed if levels fall or diarrhoea develops | Monthly checks for two months after the last antibody dose |
| **Skin, nails, eyes and breathing** NCCP 00585 | Discuss existing skin, eye or lung problems | Start the prescribed skin-care plan; report reactions early | Review skin, nails, diarrhoea and new symptoms | Eye pain, worsening breathing or severe rash needs prompt assessment | Continue care until symptoms and mineral levels settle |
| **Bowel symptoms and hydration** NCCP 00585 | Have a written diarrhoea plan and emergency contact | Tell the nurse about cramps, sweating or diarrhoea during infusion | Delayed diarrhoea can start after the first day; act using your written plan | Fever, reduced urine, severe pain or inability to drink needs urgent advice | Restart decisions require recovery and team review |
| **Fluorouracil safety and line** NCCP 00585 | Confirm the DPD test and safe line/pump arrangements | Check mouth, bowel and pump symptoms early | Review line, pump and previous-cycle toxicity | Report chest pain or unexpectedly severe early side effects immediately | Confirm disconnection and ongoing line care |
| **Cancer response and treatment choices** NCCP 00585 | Record baseline disease and agree review plan | Bring forward review if symptoms worsen | Scans and other tests follow your consultant's treatment plan | Discuss benefit, side effects and whether the goal has changed | Agree continuation, a break, another treatment or supportive care |
| **Conditional heart assessment** NCCP 00585 | ECG if cardiac function is compromised | Report new chest pain or breathing change | Emergency assessment for chest pain, collapse or severe breathlessness | Additional cardiac tests when clinically indicated | Follow any identified problem; no routine fixed echo series |

| Care / check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
| --- | --- | --- | --- | --- | --- |
| **INR if taking warfarin**<br>NCCP 00585 | Tell the team about warfarin | Check INR as clinically indicated | Report bleeding and major changes in food intake | Warfarin doses are adjusted by the prescribing team | Continue the individually agreed anticoagulation plan |

# What each check is for

1

### A 14-day reference cycle

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00585

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35